MASTER OF MARRIAGE AND FAMILY THERAPY

Fundamentals of Marriage and Family Therapy

MFT

701

A Sixteen-Week Graduate Seminar

COURSE DECK

Weeks 1 – 16

THEORY · PRACTICE · SUPERVISION · PROFESSIONAL IDENTITY

FIRST SEMESTER

1.71 UNITS

16 WEEKS · 77 SLIDES

A discipline of relationship, not a technique of repair

This course introduces marriage and family therapy as a distinctive discipline: a way of understanding distress as it is produced, maintained and transformed in relationships. Students encounter the field's intellectual history, its major theoretical traditions, and the practical and ethical demands of clinical work with couples and families. The emphasis throughout is on systemic thinking — learning to see patterns, contexts and meanings rather than isolated symptoms.

AT A GLANCE

SEMESTER	First Semester
CREDIT	1.71 units
DURATION	16 weeks · 4 phases
STRUCTURE	Lecture · seminar · skills lab
ASSESSMENT	Formulation · exam · participation

AIM OF THE COURSE

- Establish a systemic vocabulary for describing relational problems.
- Map the major models of family therapy and their points of divergence.
- Practise core clinical skills: joining, assessment, formulation, intervention.
- Locate practice within ethical, cultural and legal frameworks.
- Begin a reflective professional identity grounded in evidence and supervision.

WHAT MAKES MFT DISTINCT

- The relational system, not the individual, is the unit of assessment.
- Symptoms are understood as maintained by current interaction, not only by history.
- The therapist is a participant in the system, not an external observer.
- Change is evaluated in relationships — and through the client's own feedback.

ON COMPLETION

What you will be able to do

Six outcomes frame the course. Each week names the subset it develops; the final portfolio invites you to evidence all six.

01

Explain the systemic epistemology that distinguishes MFT from individually oriented psychotherapy.

02

Compare the major models of family therapy — structural, strategic, intergenerational, experiential, cognitive-behavioural, solution-focused and narrative.

03

Conduct a first session: join a multi-person system, negotiate consent, and elicit each member's description of the problem.

04

Construct a three-generation genogram and use circular questioning to assess relational patterns.

05

Formulate a case systemically, integrating developmental, cultural, ethical and risk considerations.

06

Use routine outcome monitoring and reflective practice to evaluate and improve your own clinical work.

Mapping · each week names the outcomes it develops, so you can trace your own progress across the sixteen weeks.

The shape of the semester

Four phases, each a distinct mode of learning: foundations, theoretical cartography, clinical encounter, then integration.

WEEK 1 · A SIXTEEN-WEEK PROGRESSION · WEEK 16

PHASE I

Foundations

Weeks 1–4

1. What is MFT?
2. Cybernetics & the systemic lens
3. A short history of the field
4. Structural & strategic foundations

PHASE II

Theoretical Cartography

Weeks 5–8

5. Intergenerational & contextual
6. Experiential & humanistic
7. Postmodern & constructivist
8. Integrative formulation

PHASE III

The Clinical Encounter

Weeks 9–12

9. First session & alliance
10. Assessment & genograms
11. Ethics, law & governance
12. Culture, power & location

PHASE IV

Integration & Transition

Weeks 13–16

13. Children & adolescents
14. Couples & attachment
15. Transitions & crisis
16. Evidence & professional identity

HOW EACH WEEK WORKS

Read, wrestle, practise, reflect

Each week follows the same four-beat structure. The deck is a teaching instrument, not a transcript: the slides carry the argument, the seminar does the work.

01

Read

Two set readings frame the week. Read for argument, not coverage: what does this model see, and what does it miss?

BEFORE THE LECTURE

02

Wrestle

The lecture introduces the model and its clinical logic. Expect to compare it with the models already in the room.

LECTURE · 2 HOURS

03

Practise

Seminar and skills lab: role-play, live supervision, genogram construction, case formulation in small groups.

SEMINAR · 2 HOURS

04

Reflect

A weekly journal entry: what did this week ask of you, personally and professionally?

JOURNAL · 20 MINUTES

Preparation is not optional: seminar time is spent applying the reading, not summarising it.

ASSESSMENT

How the course is examined

Assessment is cumulative and clinical in character: you are asked to show that you can think systemically, write clearly, and defend a formulation.

COMPONENT	FORMAT	WEIGHT
Integrative case formulation	2,500 words · one family, three models, a defended intervention plan	30%
Seminar participation	Prepared contribution, role-play, peer feedback	15%
Mid-semester examination	Short answers and a seen case	20%
Final examination	Essay questions across the full syllabus	35%

- *Feedback before grades*
- *Formulation over recall*
- *Ethical reasoning evidenced*

Foundations

Learning to see systems: the history, the cybernetic grammar, and the first maps of family structure.

01 What is MFT?

02 Cybernetics & the systemic lens

03 A short history of the field

04 Structural & strategic foundations

What Is Marriage and Family Therapy?

ESSENTIAL QUESTION

When the unit of treatment is a relationship, what exactly is it that changes?

01 · LECTURE

The relational unit of treatment

02 · DIAGRAM

The family is nested, not isolated

03 · CLINICAL CASE

The referral that reframes the problem

KEY TERMS

systemic epistemology · relational unit · presenting problem · professional identity

Key reading · Nichols & Davis (2020), ch. 1 · Becvar & Becvar (2018), ch. 1

THIS WEEK

01 Distinguish systemic practice from individually oriented psychotherapy.

02 Describe the relational unit of treatment and its implications.

03 Begin a personal statement of professional identity.

The relational unit of treatment

MFT begins from a deceptively simple claim: problems are not located only inside people. They are located in the patterns between people.

01

A DIFFERENT UNIT OF ANALYSIS

Individual therapy treats the person as the locus of difficulty. MFT treats the relationship as the client — the recurring patterns that organise how family members think, feel and act.

- The symptom is also a message about the system
- Assessment includes the family's own explanation
- Change is measured in relationships, not only symptoms

02

WHAT MFT IS NOT

The field is often reduced to "seeing the whole family". That is a setting, not a theory.

- Not the absence of diagnosis
- Not a denial of biology, trauma or individual experience
- Not the assumption that families are always the cause

03

WHY IT MATTERS

The unit of treatment decides what counts as data, who is invited in, and what improvement looks like.

- Who is present shapes what can be said
- Relational change can precede symptom change
- Families are resources, not only risks

WORKING DEFINITION

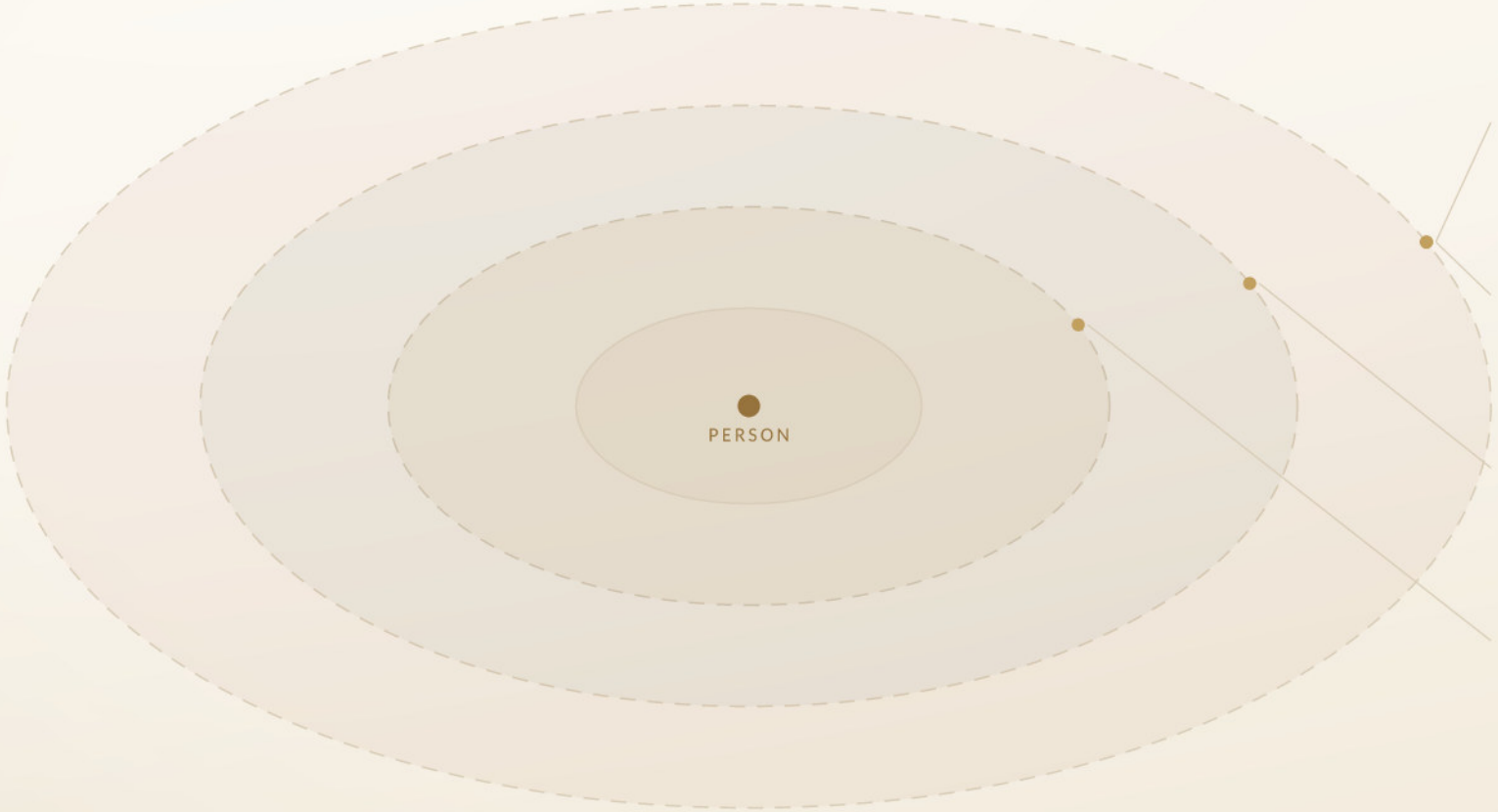
A discipline of relationship

Marriage and family therapy is the assessment and treatment of relational systems, using theories and methods that treat interaction as the primary site of change.

Nichols & Davis (2020); Becvar & Becvar (2018).

The family is nested, not isolated

A family is never only itself. It is held by kin, community, institutions and policy — each of which can carry a problem or a resource.



Culture, economy & policy

values, migration, employment, housing, law

Institutions

school, health care, workplace, courts

Extended kin & community

grandparents, kin networks, faith communities

Family & household

roles, rules, coalitions, routines

the relational unit of treatment

Adapted from Bronfenbrenner's ecological model; family systems add the dimension of meaning.

The referral that reframes the problem

CLINICAL VIGNETTE

A 14-year-old boy is referred by his school for "defiance and falling grades". His mother brings him alone. She says, "He is the problem — he won't listen." In the first session you learn that his father has worked away for a year, and that the boy now sleeps in his mother's room.

APPLY THIS WEEK

- systemic epistemology
- relational unit
- presenting problem
- professional identity

REFLECTION

01

What is the presenting problem — and what might it be a solution to?

02

Who is absent from the room, and what does their absence do?

03

What would you need to ask before forming a hypothesis?

Discuss in seminar · bring one formulation to supervision.

Cybernetics and the Systemic Lens

ESSENTIAL QUESTION

What kind of explanation does a system require — and what kind of cause is a pattern?

01 · LECTURE

From cause to loop

02 · DIAGRAM

The sequence is the unit of analysis

03 · SEMINAR

Punctuating a familiar argument

THIS WEEK

01 Explain negative and positive feedback in family interaction.

02 Distinguish first-order from second-order cybernetics.

03 Use punctuation to reframe a presenting complaint.

KEY TERMS

feedback · homeostasis · circular causality · punctuation · second-order cybernetics

Key reading · Watzlawick, Beavin & Jackson (1967), ch. 1–2 · Bateson (1972), "Cybernetics of self"

From cause to loop

Cybernetics gave family therapy its grammar: systems are defined by information fed back into them, not by a single cause acting from outside.

01

FEEDBACK

Negative feedback maintains a set point; positive feedback amplifies deviation. Both are orderly — and both can hold a symptom in place.

- A thermostat corrects deviation
- An escalating quarrel amplifies it
- A symptom can stabilise a system and escalate a conflict

02

FIRST AND SECOND ORDER

First-order cybernetics assumes an observer outside the system. Second-order cybernetics places the observer inside: the therapist is part of what they describe.

- Objectivity in parentheses
- The therapist's presence is data
- Hypotheses are interventions

03

PUNCTUATION

Members punctuate the same sequence differently: "I shout because you withdraw" / "I withdraw because you shout". Both are true; neither is the cause.

- Every punctuation implies a culprit
- Reframing changes the available next move
- The therapist punctuates too

MATURANA & VARELA

Objectivity in parentheses

We cannot observe a system without influencing it. The therapist's presence is part of the system under observation.

Watzlawick, Beavin & Jackson (1967); Bateson (1972).

The sequence is the unit of analysis

A circular map replaces the question “who started it?” with a more useful question: “what keeps it going?”



Punctuation

Each member sees their own response as reasonable and the other’s as the cause. The therapist’s first move is to punctuate the sequence, not to assign blame.

Clinical implication

Ask “what happens next?” rather than “why did you do that?” Map the loop before attempting to change it.

Map the loop before attempting to change it.

Punctuating a familiar argument

Work in pairs. Choose an argument you both know well — from your family, your workplace, or the clinic.

01 Write the argument as a loop with no beginning and no villain.

02 What does each person's punctuation protect them from feeling?

03 Where does "more of the same" appear in the sequence?

04 What would it mean to intervene at the level of the loop?

05	SILENT WRITING	10	PAIRS	20	PLENARY	05	CLOSE	MINUTES

A Short History of the Field

ESSENTIAL QUESTION

How did a profession learn to see relationships — and what did it cost to learn?

01 · LECTURE

Born at the edges of three traditions

02 · DIAGRAM

Seven moments that made the field

03 · CLINICAL CASE

Reading the present through the past

THIS WEEK

01 Trace the lineages that produced contemporary MFT.

02 Explain how the double-bind hypothesis shaped the field.

03 Situate evidence-based practice within the field's history.

KEY TERMS

child guidance · Palo Alto · double bind · professionalisation · evidence-based practice

Key reading · Nichols & Davis (2020), ch. 2–3 · Broderick & Schrader (1991), history of the profession

Born at the edges of three traditions

MFT did not begin as a single school. It assembled itself from child guidance, marriage counselling, and a research group studying communication in schizophrenia.

01

1940S–1950S · SEPARATE STREAMS

Child guidance clinics treated the child in context; marriage counselling addressed the couple. Both sat at the margin of a psychiatry organised around the individual.

02

1959 · THE DOUBLE BIND

Bateson's Palo Alto group described contradictory injunctions in families of people diagnosed with schizophrenia. The theory is now read as an early model of relational communication — and a caution about blaming families.

03

1960S–1970S · MODELS PROLIFERATE

Structural, strategic, intergenerational, experiential and systemic models emerged, each with its own account of what maintains a problem.

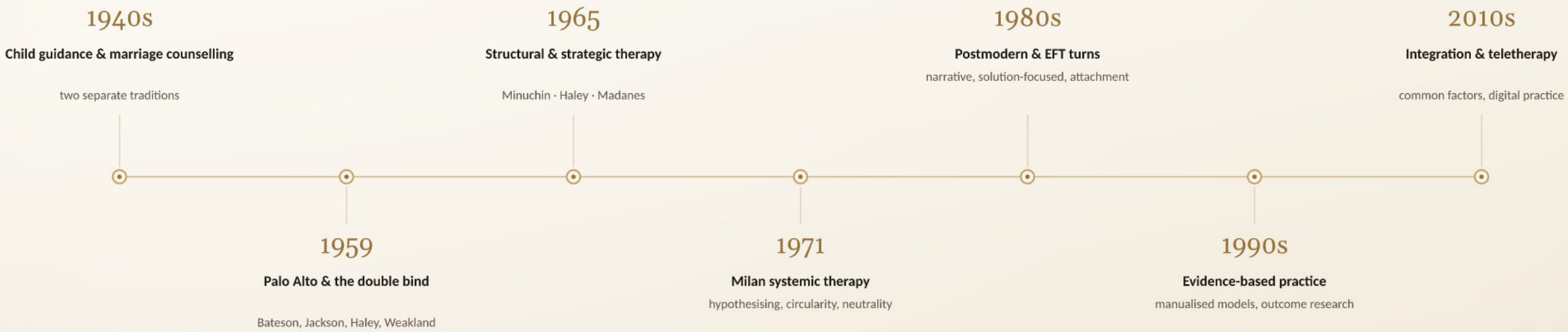
04

1980S–PRESENT · PROFESSION AND EVIDENCE

Licensure, accreditation and outcome research — alongside sustained critique from feminist, multicultural and postmodern clinicians.

Seven moments that made the field

A selective genealogy, not a complete history: each moment changed what therapists thought they were looking at.



A field born at the intersection of cybernetics, child guidance, and the clinic.

Dates are approximate and contested; the point is lineage, not chronology.

Reading the present through the past

CLINICAL VIGNETTE

A clinic introduces a manualised parenting programme for every family referred with "behaviour problems". Clinicians report that families disengage after two sessions. The service manager asks you, as the new MFT, why.

APPLY THIS WEEK

- child guidance
- Palo Alto
- double bind
- professionalisation

REFLECTION

01

Which historical tension does this reproduce?

02

What is lost when a model is applied without systemic assessment?

03

How would you argue for relational assessment without abandoning evidence?

Discuss in seminar · bring one formulation to supervision.

Structural and Strategic Foundations

ESSENTIAL QUESTION

If the problem lives in the organisation of the family, what exactly do we change?

01 · LECTURE

The architecture of family life

02 · DIAGRAM

A family, drawn as a system

03 · CLINICAL CASE

A family that cannot agree on the problem

KEY TERMS

boundary · hierarchy · subsystem · enactment · directive

Key reading · Minuchin (1974), ch. 1–4 · Haley (1976) · Minuchin & Fishman (1981)

THIS WEEK

01 Map boundaries, hierarchy and coalitions in a family.

02 Distinguish structural from strategic clinical logic.

03 Design and debrief one enactment.

The architecture of family life

Structural therapy reads the family as an organised body: who is close to whom, who holds authority, and where the boundaries fall.

01

BOUNDARIES

Clear boundaries permit both closeness and separateness. Enmeshed boundaries dissolve autonomy; disengaged boundaries isolate.

- Who knows what about whom?
- Where does a conversation stop being private?
- Which boundary is too thin, and which too thick?

02

HIERARCHY AND SUBSYSTEMS

Parents need to exercise authority as a subsystem. Children need access to the sibling subsystem — not to the parental one.

- Spousal subsystem
- Parental subsystem
- Sibling subsystem

03

COALITIONS

A cross-generational coalition — one parent allied with a child against the other — is a structural signature of many presenting problems.

- Who is recruited into which conflict?
- What does the coalition protect?
- What would a stronger dyad make possible?

MINUCHIN

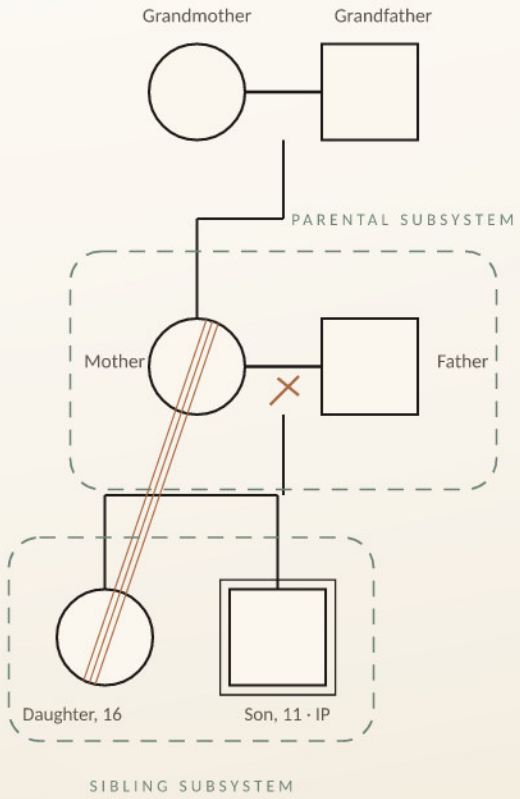
The map is not the family

A structural map is a working hypothesis, drawn to be revised. It is a clinical instrument, not a verdict on the family.

Minuchin (1974); Haley (1976).

A family, drawn as a system

Boundaries, hierarchy, coalition and symptom appear in a single view. The genogram is a hypothesis you can test in the room.



Reading the map

- male
- female
- deceased
- marriage
- conflict
- enmeshment
- subsystem boundary
- index patient

Conventions after McGoldrick, Gerson & Petry (2008).

Symbols follow McGoldrick, Gerson & Petry (2008).

A family that cannot agree on the problem

CLINICAL VIGNETTE

A mother complains that her husband "undermines" her with their 15-year-old daughter. The father says the mother is "too strict". The daughter says nothing. When the parents begin to argue, the daughter interrupts with a headache and the argument stops.

APPLY THIS WEEK

- boundary
- hierarchy
- subsystem
- enactment

REFLECTION

01

Draw the subsystem boundaries and the triangle.

02

What is the daughter's headache doing in the system?

03

Which dyad would you strengthen first — and why?

Discuss in seminar · bring one formulation to supervision.

Theoretical Cartography

The major traditions, held side by side — intergenerational, experiential, postmodern, and the discipline of integration.

05 Intergenerational & contextual

06 Experiential & humanistic

07 Postmodern & constructivist

08 Integrative formulation

Intergenerational and Contextual Approaches

ESSENTIAL QUESTION

How does the past continue to speak in the present — and who is loyal to whom?

01 · LECTURE

The emotional process that travels

02 · DIAGRAM

The triangle as the unit of emotional process

03 · SEMINAR

The ledger of loyalty

KEY TERMS

differentiation · triangle · multigenerational transmission · legacy · loyalty

Key reading · Bowen (1978), ch. 1–3 · Kerr & Bowen (1988) · Boszormenyi-Nagy & Spark (1973)

THIS WEEK

01 Explain differentiation of self and triangulation.

02 Construct and interpret a three-generation genogram.

03 Apply the concepts of legacy, entitlement and loyalty.

The emotional process that travels

Bowen shifted attention from the presenting symptom to the emotional process that moves through generations — and to the therapist's own family as an instrument of understanding.

01

DIFFERENTIATION OF SELF

The capacity to stay connected while maintaining one's own thinking. Low differentiation produces fusion, reactivity and the feeling that one cannot survive the other's disapproval.

- Thinking versus feeling
- Solid self versus pseudo-self
- The goal is connection, not distance

02

TRIANGLES

Under tension, dyads recruit a third. Triangles distribute anxiety — and freeze conflict in place.

- Who is recruited when tension rises?
- Which side carries the symptom?
- Can each dyad be strengthened without the third?

03

LEGACY AND LOYALTY

Boszormenyi-Nagy added an ethical dimension: families keep a ledger of give-and-take. Entitlement, loyalty and invisible debts shape what members feel they owe.

- What was handed down that was not chosen?
- What loyalty would change require?
- Where is fairness being negotiated?

A CAUTION

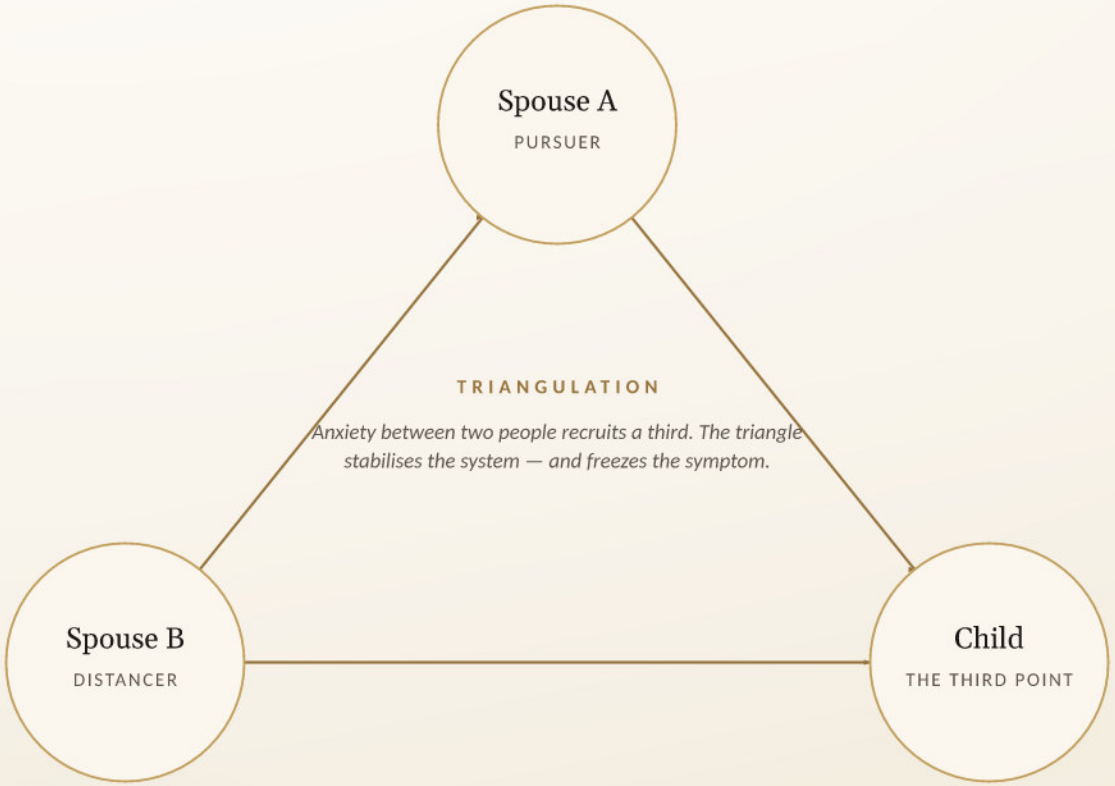
Explaining is not excusing

Intergenerational work describes transmission. It does not absolve responsibility, and it must never be used to blame a previous generation for a present choice.

Bowen (1978); Kerr & Bowen (1988); Boszormenyi-Nagy & Spark (1973).

The triangle as the unit of emotional process

Bowen's smallest stable relational system is not the dyad but the triangle. Anxiety moves; the third person absorbs it.



What the therapist watches

1. Who is drawn in when tension rises?
2. Which side of the triangle carries the symptom?
3. Can each dyad be strengthened without the third?

Detriangling begins with the therapist staying out of the triangle.

After Bowen (1978); see also Kerr & Bowen (1988).

The ledger of loyalty

This is a reflective seminar. Speak only to what you are willing to share; the exercise is about seeing, not disclosing.

01 What was handed down to you that you did not choose?

02 What loyalty would be tested if you changed one pattern?

03 Where does “undifferentiated” appear in your own family?

04 How would you ask about legacy without pathologising it?

05	SILENT WRITING	10	PAIRS	20	PLENARY	05	CLOSE	MINUTES

Experiential and Humanistic Traditions

ESSENTIAL QUESTION

What happens when the therapist stops explaining and starts experiencing?

01 · LECTURE

The person of the therapist

02 · DIAGRAM

Four stances, one cost each

03 · CLINICAL CASE

The family that explains everything

KEY TERMS

congruence · sculpting · immediacy · symbolic-experiential · self-of-the-therapist

Key reading · Satir (1967) · Whitaker (1989) · Greenberg & Johnson (1988)

THIS WEEK

01 Describe Satir's communication stances and the goal of congruence.

02 Explain the symbolic-experiential use of the therapist's self.

03 Practise immediacy without shaming.

The person of the therapist

Experiential models insist that change happens in the encounter — not in the explanation of the encounter.

01

SATIR · COMMUNICATION AND CONGRUENCE

Stances are survival strategies learned early: placating, blaming, being super-reasonable, being irrelevant. Congruence is the goal — the body, the words and the feeling agreeing.

- Sculpting makes structure visible in space
- The therapist models congruence rather than teaching it
- Family maps trace three generations of coping

02

WHITAKER · SYMBOLIC-EXPERIENTIAL

Therapy trusts the family's own process. The therapist joins, provokes, plays and uses their own experience as part of the field.

- The therapist's growth is part of the treatment
- Symptoms can be understood symbolically
- Play is a serious clinical method

03

THE EMOTION-FOCUSED LINEAGE

Greenberg and Johnson moved experiential work toward affect regulation and attachment, making it researchable without losing its human centre.

- Emotion as information
- Therapeutic presence
- A bridge to process research

WHITAKER

The self as instrument

Personal therapy and supervision are not extras added to training. They are the means by which the therapist becomes usable.

Satir (1967); Whitaker (1989); Greenberg & Johnson (1988).

Four stances, one cost each

Each stance solves an immediate problem and purchases a longer one. The clinical task is to make the cost visible without contempt.

01



Placator

"Whatever you want."

Self erased to keep the peace.

02



Blamer

"It is all your fault."

Connection traded for control.

03



Super-reasonable

"Let us be logical."

Feeling exiled from the room.

04



Irrelevant

"Anyway — did you see…?"

The real subject never lands.

The goal is not a fifth stance but congruence — the body, the words and the feeling saying the same thing.

After Virginia Satir's communication stances.

The family that explains everything

CLINICAL VIGNETTE

A couple arrives with a spreadsheet of grievances. Each has rehearsed their account. When you ask what each felt during the argument, both answer with the other's motives. The session is calm, articulate, and completely stuck.

APPLY THIS WEEK

- congruence
- sculpting
- immediacy
- symbolic-experiential

REFLECTION

01

What is the function of the explanation?

02

How would you move from story to affect without shaming?

03

What would immediacy look like here?

Discuss in seminar · bring one formulation to supervision.

Postmodern and Constructivist Turns

ESSENTIAL QUESTION

If meaning is made in language, what is therapy making?

01 · LECTURE

Meaning as the material of therapy

02 · DIAGRAM

Two epistemologies, held side by side

03 · CLINICAL CASE

Externalising a diagnosis

KEY TERMS

social constructionism · externalising · exception · not-knowing · reflecting team

Key reading · White & Epston (1990), ch. 2–4 · de Shazer (1985) · Andersen (1991)

THIS WEEK

01 Compare first-order systemic and postmodern practice.

02 Describe solution-focused and narrative methods.

03 Use an externalising question in a role-play.

Meaning as the material of therapy

Postmodern practice shifted the therapist's authority from diagnosis to curiosity — and made the family's language the central clinical material.

01

MILAN SYSTEMIC

Hypothesising, circularity and neutrality: the team's ideas are provisional, and difference is the engine of change.

- Hypotheses are held, not owned
- Circular questions reveal difference
- Neutrality is curiosity in action

02

SOLUTION-FOCUSED

Problems have exceptions; change is already occurring. The miracle question, scaling and exception-finding orient the family toward what already works.

- Assume competence
- Amplify exceptions
- Small change is enough to begin

03

NARRATIVE

Problems are stories, not essences. Externalising separates the person from the problem; unique outcomes open a preferred story.

- "The anxiety visits you" rather than "you are anxious"
- Deconstruct the dominant story
- Re-author in the family's own language

CRITIQUE

Not-knowing is not ignorance

The therapist remains accountable for power, ethics and outcome. Curiosity does not suspend responsibility for safety.

Selvini Palazzoli et al. (1978); de Shazer (1985); White & Epston (1990); Andersen (1991).

Two epistemologies, held side by side

The distinction is not a war. Most contemporary practitioners move between these positions within a single session.

	First-order systemic	Postmodern / constructivist
LOCUS OF MEANING	In structure and interaction patterns observable by the therapist	In language, story and relationship; meanings are co-created
THERAPIST STANCE	Expert observer and agent of change	Curious co-author; not-knowing
USE OF DIAGNOSIS	Descriptive; guides systemic intervention	Provisional; risks totalising the family
MECHANISM OF CHANGE	Restructure, reframe, interrupt sequences	Deconstruct, re-author, open new meanings
SIGNATURE METHODS	Enactment, directives, genograms, boundaries	Reflecting teams, externalising, miracle question

After Cecchin (1987) and White & Epston (1990).

Externalising a diagnosis

CLINICAL VIGNETTE

Parents describe their 10-year-old as "the ADHD child". School reports dominate dinner-table conversation. The boy says, "I'm the naughty one."

APPLY THIS WEEK

- social constructionism
- externalising
- exception
- not-knowing

REFLECTION

01

What might it mean to externalise ADHD without dismissing it?

02

What unique outcomes exist in the family's own account?

03

Where does the family's preferred story begin?

Discuss in seminar · bring one formulation to supervision.

Integrative Case Conceptualisation

ESSENTIAL QUESTION

How does a therapist choose a theory — and then hold it lightly?

01 · LECTURE

Formulation as a disciplined hypothesis

02 · DIAGRAM

One family, four formulations

03 · SEMINAR

Same family, four lenses

KEY TERMS

formulation · hypothesis · maintaining pattern · integration

Key reading · Carr (2019), ch. 4 · Eells (2015), case formulation

THIS WEEK

01 Compare formulations of one family across four models.

02 Write a systemic formulation that includes risk and context.

03 Defend a reasoned choice of intervention.

Formulation as a disciplined hypothesis

A formulation is not a diagnosis with a story attached. It is a testable account of how a problem is maintained, in this system, at this moment.

01

WHAT A FORMULATION MUST DO

Explain the presenting problem, identify maintaining patterns, locate strengths, specify a point of leverage, and anticipate risk.

- Description before explanation
- Maintaining patterns, not only origins
- A plan that can be revised

02

FOUR PS, SYSTEMICALLY

Predisposing, precipitating, perpetuating and protective factors — each located in the family's relational and social context.

- Predisposing · history and temperament
- Precipitating · events and transitions
- Perpetuating · cycles, coalitions, meanings

03

HOLDING THEORY LIGHTLY

Different models illuminate different surfaces. The question is not which model is true, but which formulation opens the most useful next step.

- Use more than one lens
- Let the family's feedback correct you
- Integration is a discipline, not a blend

ASSESSMENT

Formative case formulation

Due in Week 9. One family, three formulations and a defended intervention plan, 2,500 words.

Carr (2019); Eells (2015).

One family, four formulations

The same presenting problem yields four different accounts of what maintains it — and four different first moves.

MODEL	FOCUS OF FORMULATION	MECHANISM OF THE PROBLEM	SIGNATURE INTERVENTION
Structural	Boundaries, hierarchy, subsystems	Enmeshed or disengaged boundaries; cross-generational coalitions	Enactment, boundary-making, unbalancing
Strategic	Attempted solutions maintaining the problem	Circular feedback; power in the symptom	Directives, paradoxical tasks, reframing
Intergenerational	Differentiation and multigenerational transmission	Triangulation; unresolved legacy and loyalty	Genogram, detriangling, coaching the dyad
Narrative	Dominant stories and their effects	Problem-saturated identity; internalised deficit	Externalising, unique outcomes, re-authoring

A comparative grid for the Week 9 written formulation.

Same family, four lenses

Using the family from your case formulation, argue the case for each model in turn — then choose one and justify the choice.

01 What does each model see that the others miss?

02 Where do the formulations contradict one another?

03 Which intervention follows from your chosen account?

04 What would count as evidence that you were wrong?

05	SILENT WRITING	10	PAIRS	20	PLENARY	05	CLOSE	MINUTES

The Clinical Encounter

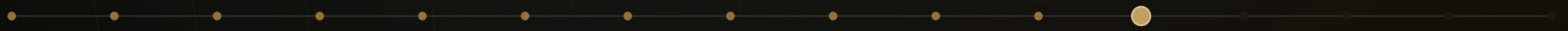
Joining, assessing and negotiating the ethical and cultural complexity of real clinical work.

09
First session & alliance

010
Assessment & genograms

011
Ethics, law & governance

012
Culture, power & location



The First Session and the Therapeutic Alliance

ESSENTIAL QUESTION

How do you join a system without joining its triangles?

01 · LECTURE

Joining before intervening

02 · DIAGRAM

The alliance is not a dyad

03 · CLINICAL CASE

The first ten minutes

KEY TERMS

joining · alliance · neutrality · informed consent · split alliance

Key reading · Minuchin & Fishman (1981), ch. 1–2 · Friedlander, Escudero & Heatherington (2006)

THIS WEEK

01 Structure a first session from opening to next steps.

02 Build and monitor a multi-person alliance.

03 Negotiate consent and confidentiality with several clients.

Joining before intervening

The first session is already an intervention. It establishes who is heard, what is named as the problem, and whether change feels possible.

01

THE TASKS OF SESSION ONE

Engage every member, establish safety and consent, elicit each description, observe sequences, offer a tentative formulation, agree next steps.

- Who speaks first — and who is spoken for?
- What is the family's own explanation?
- What has already been tried?

02

THE MULTI-PERSON ALLIANCE

In couple and family work the alliance is not one relationship but several — and they can conflict.

- Bond, goals, tasks
- Split alliances reproduce triangles
- Rupture is data, not failure

03

NEUTRALITY, REVISED

Neutrality is not indifference. It is a commitment not to become the spokesperson for any one member — including the most articulate one.

- Curiosity over alignment
- Multidirectional partiality
- The absent member is still in the room

CONSENT

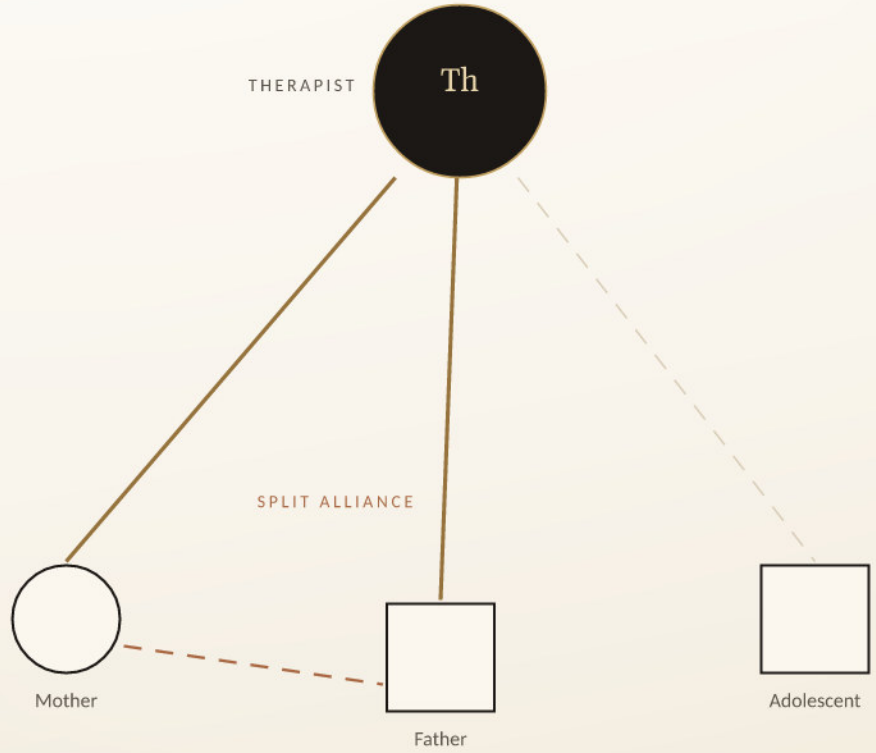
Who is the client?

With multiple clients, consent must specify who holds confidentiality, how records are shared, and what happens when members disagree about disclosure.

Minuchin & Fishman (1981); Pinsof & Wynne (1995).

The alliance is not a dyad

Every statement is heard by more than one person — and heard differently. The therapist's task is to remain available to all of them.



Line weight indicates strength of alliance.

After Friedlander, Escudero & Heatherington (2006).

The alliance is not a dyad

Triadic by nature

Every statement is heard by more than one person, and heard differently.

Split alliances

A strong bond with one member, paid for by distance from another, reproduces the family's own triangle.

Repair in session

Name the rupture, validate both positions, and re-establish the therapist as available to all.

The first ten minutes

CLINICAL VIGNETTE

A couple arrives furious. One wants to leave; the other booked the session secretly. Both look at you before they sit down.

APPLY THIS WEEK

- joining
- alliance
- neutrality
- informed consent

REFLECTION

01

What do you say in the first minute?

02

How do you handle the secret booking?

03

What would make both feel heard without agreeing with either?

Discuss in seminar · bring one formulation to supervision.

10

Assessment, Genograms and Information Gathering

ESSENTIAL QUESTION

What must a therapist know before intervening — and what does the asking itself do?

01 · LECTURE

Assessment as a relational act

02 · DIAGRAM

Three generations in one view

03 · CLINICAL CASE

The family who would not draw

KEY TERMS

genogram · circular questioning · interventive interviewing · risk assessment

Key reading · McGoldrick, Gerson & Petry (2008), ch. 1–3 · Tomm (1988), interventive interviewing

THIS WEEK

01 Construct a three-generation genogram with a family.

02 Use circular questions to elicit relational difference.

03 Integrate risk screening into systemic assessment.

Assessment as a relational act

Every question is an intervention: it directs attention, invites a description, and positions the family in relation to the problem.

01

THE GENOGRAM

Three generations, drawn with the family, in the first two sessions. The drawing matters less than the conversation it makes possible.

- Structure · household, marriages, births, deaths, migration
- Patterns · repetition, cut-offs, coalitions
- Meaning · names, legacies, secrets

02

CIRCULAR QUESTIONING

Questions about difference and connection: "Who is most worried? Who shows it least? What does your mother do when your father goes quiet?"

- Ask about relationships, not individuals
- Ask each member to compare
- Let the question land before asking the next

03

RISK AND SAFEGUARDING

Ask directly, document clearly, consult, and follow local law and agency procedure. Risk assessment is not a form; it is a conversation with consequences.

- Violence and coercive control
- Self-harm and suicidal ideation
- Child protection and elder safety

TOMM

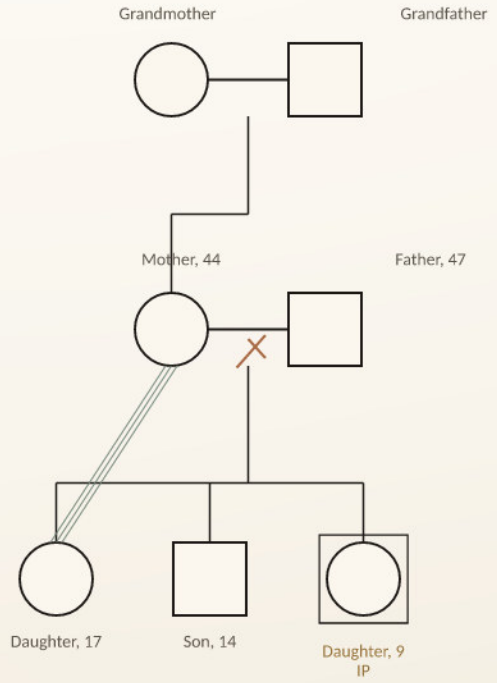
Questions have effects

Interventive interviewing treats the question as a clinical move: it creates difference, opens possibility, or closes a conversation.

McGoldrick, Gerson & Petry (2008); Tomm (1988); Fleuridas, Nelson & Rosenthal (1986).

Three generations in one view

A genogram turns history into a hypothesis: where the family repeats, where it cuts off, and where the symptom sits.



Genogram conventions

- male
- female
- index patient
- deceased
- marriage
- conflict
- close / fused
- cut-off

Three generations, one view: pattern, not blame.

Conventions after McGoldrick, Gerson & Petry (2008).

The family who would not draw

CLINICAL VIGNETTE

The parents decline the genogram: "We don't talk about the past." The 17-year-old asks whether their uncle is "the one nobody mentions".

APPLY THIS WEEK

- genogram
- circular questioning
- interventive interviewing
- risk assessment

REFLECTION

01

What is the refusal protecting?

02

How do you honour the boundary and stay curious?

03

What does the adolescent already know — and what is that knowledge doing?

Discuss in seminar · bring one formulation to supervision.

Ethics, Law and Professional Governance

ESSENTIAL QUESTION

Whose confidence is it, when the client is a family?

01 · LECTURE

The ethical complexity of the many

02 · DIAGRAM

Deciding what may be shared

03 · SEMINAR

The secret in the room

KEY TERMS

informed consent · confidentiality · duty to warn · competence · documentation

THIS WEEK

01 Apply the AAMFT Code of Ethics to multi-client dilemmas.

02 Manage confidentiality, secrets and record-sharing.

03 Document ethical decisions defensibly.

Key reading · AAMFT (2015), Code of Ethics · Corey, Corey & Callanan (2019), ch. 6–8

The ethical complexity of the many

Family therapy multiplies the number of people to whom the therapist owes a duty — and the number of ways those duties can collide.

01

CONSENT IS CONTINUOUS

Consent is not a signature at intake. It is a conversation renewed whenever the system changes — a new partner, an adolescent, a court order.

- Who is the client?
- What has each member agreed to?
- What changes when someone joins or leaves?

02

CONFIDENTIALITY IS NOT SINGULAR

Who owns the record? Who may see it? What happens to a secret disclosed by one member? These questions must be answered before they are urgent.

- Secrets within the system
- Records and third parties
- Limits required by law

03

BOUNDARIES AND COMPETENCE

Multiple relationships are endemic in small communities. Competence is an ethical obligation, not a personal achievement.

- Recognise the limits of your training
- Seek supervision early
- Document the reasoning, not only the outcome

AAMFT CODE

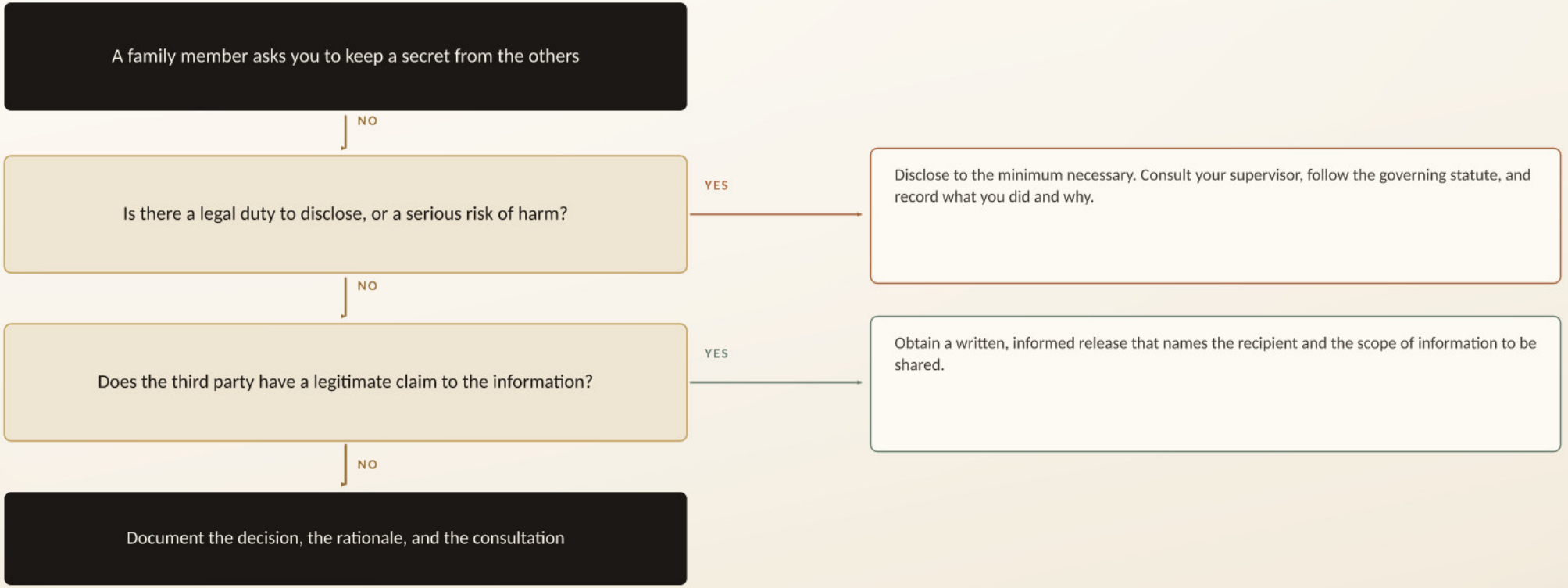
Nonmaleficence first

When duties conflict, the standard is the least harm and the most transparency achievable. Consultation is both a safeguard and an ethical practice.

AAMFT (2015); Corey, Corey & Callanan (2019).

Deciding what may be shared

A decision pathway for the most common ethical pressure in family work: the secret in the room.



Adapt to your jurisdiction, agency policy and professional code.

The secret in the room

A mother tells you, alone, that she is planning to leave her husband. She asks you not to tell him. The couple is due in twenty minutes.

01 What did you promise at intake — explicitly and implicitly?

02 What are the risks of holding the secret, and of disclosing it?

03 What would you say, and to whom, in the next session?

04 How would you document your reasoning?

05	SILENT WRITING	10	PAIRS	20	PLENARY	05	CLOSE	MINUTES

Culture, Power and Social Location

ESSENTIAL QUESTION

Whose normal is the therapy measuring against?

01 · LECTURE

Culture is not a variable; it is the water

02 · DIAGRAM

Mapping social location

03 · CLINICAL CASE

The interpreter in the room

KEY TERMS

cultural humility · intersectionality · power · cultural genogram · positionality

Key reading · Hays (2016), ch. 1–4 · McGoldrick, Giordano & Garcia-Preto (2005)

THIS WEEK

01 Apply cultural humility as a clinical stance.

02 Use the ADDRESSING framework to map social location.

03 Interrogate your own cultural assumptions in supervision.

Culture is not a variable; it is the water

Every family is cultural, including the therapist's. The question is not whether culture matters, but whose cultural assumptions are doing the work.

01

BEYOND COMPETENCE

Cultural competence risks treating culture as a checklist. Cultural humility is a stance: self-reflection, attention to power, and lifelong learning.

- Notice your own location
- Treat the family as expert on its culture
- Expect to be corrected

02

INTERSECTIONALITY AND POWER

Race, class, gender, faith, migration status and sexuality intersect. No single identity explains a family — and the therapist's institution is part of the power in the room.

- Who is speaking, and who is spoken about?
- Who is absent?
- What does the family have to risk to be understood?

03

LOCAL CONTEXT

In Nigeria and across Africa, extended kin, faith communities and communal child-rearing are central resources — and sometimes sites of obligation, surveillance and conflict.

- Kinship as support and constraint
- Faith as coping and as authority
- Migration and the transnational family

HARDY & LASZLOFFY (1995)

The cultural genogram

Map the family's cultural identity, migration, language and the meanings attached to difference — including what the family was taught to think about other groups.

Hays (2016); Falicov (2014); Hardy & Laszloffy (1995).

Mapping social location

ADDRESSING asks the therapist to locate themselves before locating the family — and to notice which differences carry power in this room.



Framework adapted from Hays (2016); see also Hardy & Laszloffy (1995) on the cultural genogram.

Working principles

1. **Self-reflection**
   Notice your own cultural location, assumptions and discomfort.

2. **Power**
   Ask who is speaking, who is being spoken about, and who is absent.

3. **Learning**
   Treat the family as the expert on its own culture; remain a learner.

Framework after Hays (2016).

The interpreter in the room

CLINICAL VIGNETTE

A family recently arrived from a rural area speaks a language you do not share. The interpreter is a member of their faith community. The parents defer to the interpreter; the daughter answers in the dominant language.

APPLY THIS WEEK

- cultural humility
- intersectionality
- power
- cultural genogram

REFLECTION

01

What are the risks of using a community interpreter?

02

How do you protect the daughter's voice?

03

What would you need to ask about the family's own explanation?

Discuss in seminar · bring one formulation to supervision.

Integration & Transition

Applying systemic thinking across the life course, and beginning the transition to supervised practice.

013
Children & adolescents

014
Couples & attachment

015
Transitions & crisis

016
Evidence & professional identity

13

Children, Adolescents and Parenting

ESSENTIAL QUESTION

How does a child's symptom speak for the family — and to it?

01 · LECTURE

The child in the system

02 · DIAGRAM

Development as a clinical frame

03 · CLINICAL CASE

The symptomatic child

KEY TERMS

parentification · developmental task · triangulation · parenting coalition

THIS WEEK

01 Conduct child-inclusive family sessions.

02 Apply a developmental frame to presenting problems.

03 Work with the parental subsystem as a coalition.

Key reading · Combrinck-Graham (2006) · Sori & Hecker (2015)

The child in the system

Children are often the reason a family comes — and the least powerful person in the room.

01

DEVELOPMENT AS CONSTRAINT

What looks like defiance at 14 may be a developmentally ordinary bid for autonomy; what looks like independence at 7 may be neglect.

- Development is relational, not only biological
- Behaviour is age-referenced
- The symptom may be on schedule

02

TRIANGULATION AND PARENTIFICATION

A child recruited into parental conflict may present the symptom. A child who cares for a parent may present competence — and neither is a diagnosis.

- Who is protecting whom?
- Who has been promoted to adult?
- What does the child's behaviour hold together?

03

PARENTING AS A SUBSYSTEM

Effective parenting requires a coalition between caregivers, not a coalition with the child. Agreement is not uniformity.

- Negotiate the frame before the rules
- Support the weaker parental voice
- Keep children out of the executive

ALLIANCE

The adolescent's confidence

Adolescents need a space that is theirs. Confidentiality must be negotiated explicitly with parents — and its limits stated in advance.

Combrinck-Graham (2006); Sori & Hecker (2015).

Development as a clinical frame

Each stage carries a task, a vulnerability and a particular way of speaking without words.

0–3	4–6	7–11	12–18	19+
Infancy & toddlerhood	Early childhood	Middle childhood	Adolescence	Emerging adulthood
Attachment, regulation, co-regulation	Language, play, initiative	School, peers, competence	Identity, autonomy, sexuality	Launching, individuation
Include caregivers; observe the dyad	Play-based methods; parent guidance	School–family liaison; structure	Negotiate privacy; alliance with teen	Re-contract the parent–child bond
CLINICAL FOCUS	CLINICAL FOCUS	CLINICAL FOCUS	CLINICAL FOCUS	CLINICAL FOCUS

Development is not a backdrop to family life — it is one of the family's organising constraints.

Read alongside the family life cycle (Week 15).

The symptomatic child

CLINICAL VIGNETTE

A 9-year-old refuses school. Her parents disagree about whether she is "manipulative" or "anxious". Her mother has recently returned to full-time work; her father works nights.

APPLY THIS WEEK

- parentification
- developmental task
- triangulation
- parenting coalition

REFLECTION

01

What developmental and family transitions converge here?

02

What is the school refusal doing for the system?

03

Who needs to be in the next session?

Discuss in seminar · bring one formulation to supervision.

Couple Relationships, Attachment and Repair

ESSENTIAL QUESTION

What if the problem is not the argument, but the cycle beneath it?

01 · LECTURE

The cycle beneath the argument

02 · DIAGRAM

Mapping the negative cycle

03 · SEMINAR

The argument underneath

KEY TERMS

negative cycle · attachment · pursuer–withdrawer · repair

Key reading · Johnson (2019), ch. 1–4 · Gottman & Gottman (2017)

THIS WEEK

01 Describe attachment cycles in couple distress.

02 Map a pursuer–withdrawer sequence.

03 Identify and rehearse a repair move.

The cycle beneath the argument

Couples do not fight about the dishes. They fight about whether they matter to each other — and the dishes are the only vocabulary available.

01

ATTACHMENT IN ADULTHOOD

Proximity-seeking, safe haven, secure base: the needs that organise infancy organise marriage. Attachment is not dependency; it is how humans regulate fear.

- Protest behaviours signal threat
- Withdrawal protects and abandons
- Security is built in small repetitions

02

THE PURSUER–WITHDRAWER CYCLE

Protest and withdrawal are two versions of the same attachment cry. Each triggers the other, and each is experienced as the cause.

- Criticism invites distance
- Distance confirms abandonment
- The cycle, not the partner, is the problem

03

REPAIR

Repair is not the absence of conflict. It is the return to connection after rupture — and it can be learned.

- Name the cycle
- Validate the fear beneath the move
- Risk a new response

EMOTIONALLY FOCUSED THERAPY

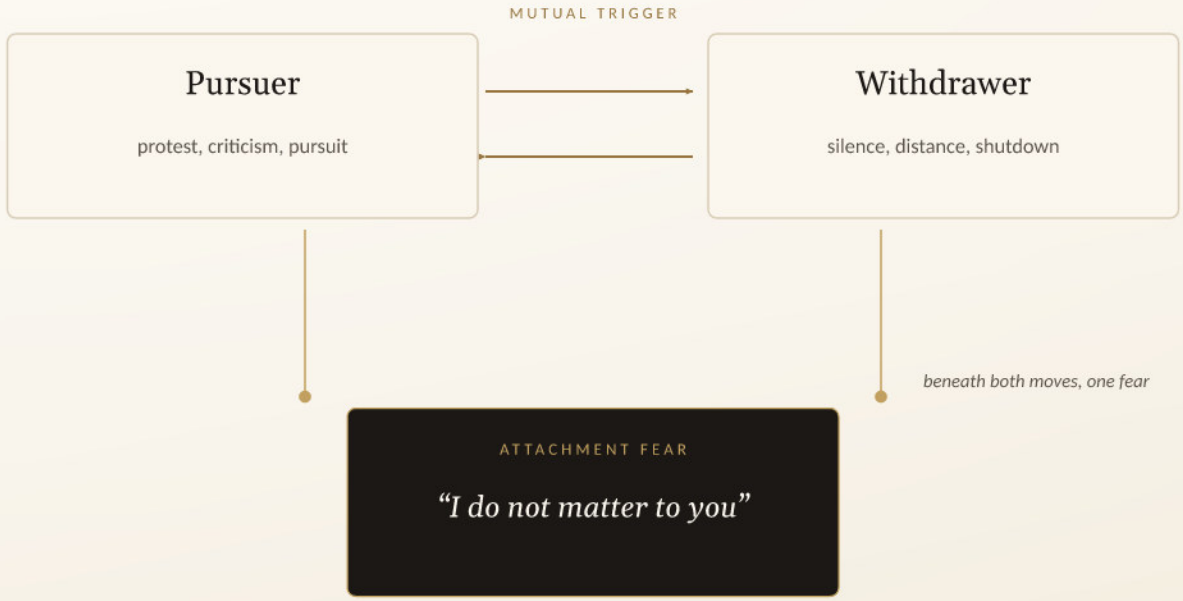
Change the cycle, not the person

EFT tracks the cycle, accesses the attachment emotion, and stages a new conversation in which each partner can be reached.

Johnson (2019); Gottman & Gottman (2017); Epstein & Baucom (2002).

Mapping the negative cycle

A three-move loop, repeated thousands of times, until it feels like the truth about the relationship.



Interrupting the cycle

1. **Name it**

   “Can we slow this down and look at the loop together?”

2. **Validate both**

   Each response makes sense given the attachment fear beneath it.

3. **Create a new move**

   Risk a softer signal; respond with accessibility and responsiveness.

After Johnson (2019).

The argument underneath

In pairs, take one repeated argument and map it as an attachment cycle rather than a disagreement.

01 What is each partner asking for beneath the complaint?

02 Which move most reliably triggers the other?

03 Where could a softer signal be risked?

04 What would make repair credible rather than polite?

05	SILENT WRITING	10	PAIRS	20	PLENARY	05	CLOSE

MINUTES

Family Transitions, Crisis and High-Conflict Contexts

ESSENTIAL QUESTION

When the family changes shape, what holds it together — and what must be made safe first?

01 · LECTURE

Symptoms at the threshold

02 · DIAGRAM

The staircase of transitions

03 · CLINICAL CASE

The transition nobody named

KEY TERMS

transition · family life cycle · ambiguous loss · coercive control · safety plan

THIS WEEK

01 Map transitions using a family life cycle frame.

02 Assess safety and crisis in family work.

03 Work systemically with separation and stepfamily formation.

Key reading · Carter & McGoldrick (1999), ch. 1–2 · Walsh (2016) · Emery (2012)

Symptoms at the threshold

Families do not present at random. Symptoms cluster at transitions — when the old rules no longer fit and the new ones do not yet exist.

01

THE FAMILY LIFE CYCLE

Stages and transitions; vertical stressors (history, legacy) meet horizontal stressors (events, time). The symptom often appears in the in-between.

- Not a timetable but a set of tasks
- Transitions renegotiate every relationship
- Cultural timing varies widely

02

CRISIS AND SAFETY

Crisis compresses time. The first tasks are safety and stabilisation; meaning-making comes after the system can hold it.

- Screen for violence separately
- Do not do couple work under coercive control
- Document and refer

03

SEPARATION AND STEPFAMILY

Divorce is not an event but a process. Co-parenting requires a new boundary around the couple relationship — and stepfamily formation requires time.

- Grief alongside relief
- Loyalty conflicts
- A slower integration than adults expect

AMBIGUOUS LOSS

Present and absent at once

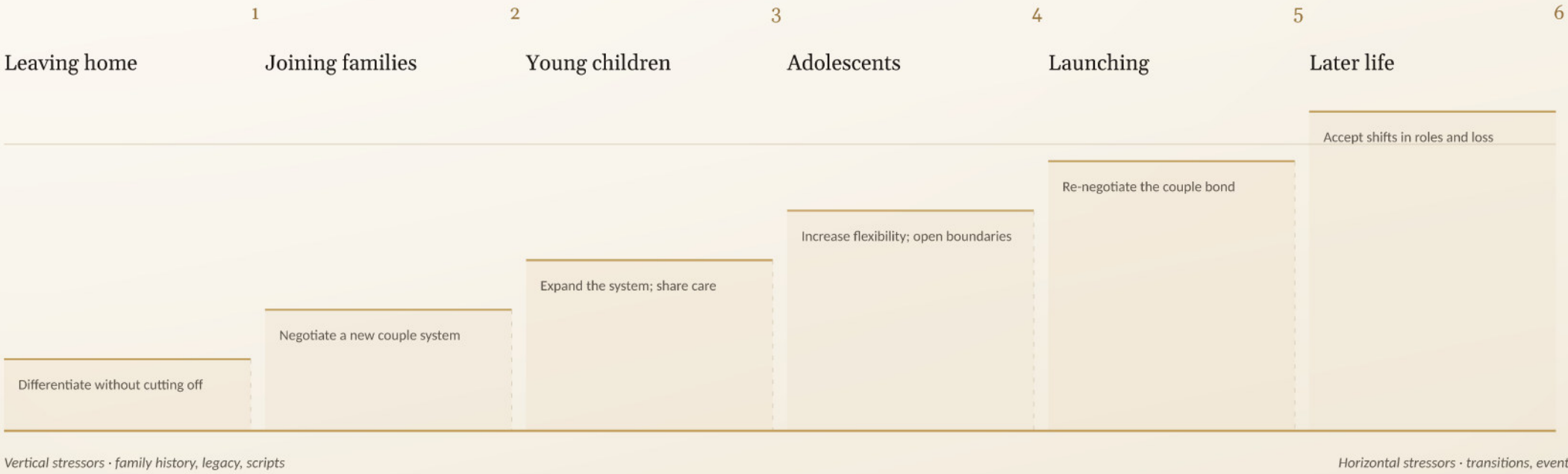
When a person is physically absent but psychologically present — or the reverse — grief cannot close. Naming the ambiguity can restore movement.

Carter & McGoldrick (1999); Walsh (2016); Emery (2012).

The staircase of transitions

Each step up requires the family to reorganise. The step down is where symptoms often appear.

The symptom often appears at the transition, not inside the stage itself.



After Carter & McGoldrick (1999).

The transition nobody named

CLINICAL VIGNETTE

A family seeks help because their 16-year-old has "become impossible". The eldest child left for university last term; the grandmother moved in after a stroke three months ago.

APPLY THIS WEEK

- transition
- family life cycle
- ambiguous loss
- coercive control

REFLECTION

01

Which transitions are stacking up?

02

What meaning does the adolescent's behaviour carry?

03

What would stabilisation look like in the first three sessions?

Discuss in seminar · bring one formulation to supervision.

Evidence, Outcomes and Professional Identity

ESSENTIAL QUESTION

How will you know your work is helping — and who are you becoming as a therapist?

01 · LECTURE

What makes therapy work

02 · DIAGRAM

Feedback as a clinical discipline

03 · SEMINAR

The clinician you are becoming

KEY TERMS

common factors · evidence-based practice · routine outcome monitoring · reflective practice

Key reading · Sprenkle, Davis & Lebow (2009) · Wampold & Imel (2015) · Duncan et al. (2010)

THIS WEEK

01 Explain common factors and evidence-based practice.

02 Use routine outcome monitoring in a session.

03 Write a professional development and supervision plan.

What makes therapy work

Models differ; the factors that carry change overlap. Evidence is not the enemy of relationship — it is a discipline of humility.

01

COMMON FACTORS

Alliance, empathy, expectancy and client resources account for substantial variance across models — which is why the therapeutic relationship is a clinical variable, not a pleasant extra.

- Alliance as outcome predictor
- Therapist effects are real
- Fit the model to the family

02

EVIDENCE-BASED PRACTICE

EBP is the integration of best research, clinical expertise and client values — not the mechanical application of a manual.

- Know the evidence base for your model
- Adapt without abandoning fidelity
- Name uncertainty honestly

03

ROUTINE OUTCOME MONITORING

Measure each session; review with the client; act early on deterioration. Feedback turns the client's view into clinical data.

- ORS and SRS
- Review the score together
- Escalate, consult, or change method

FEEDBACK-INFORMED

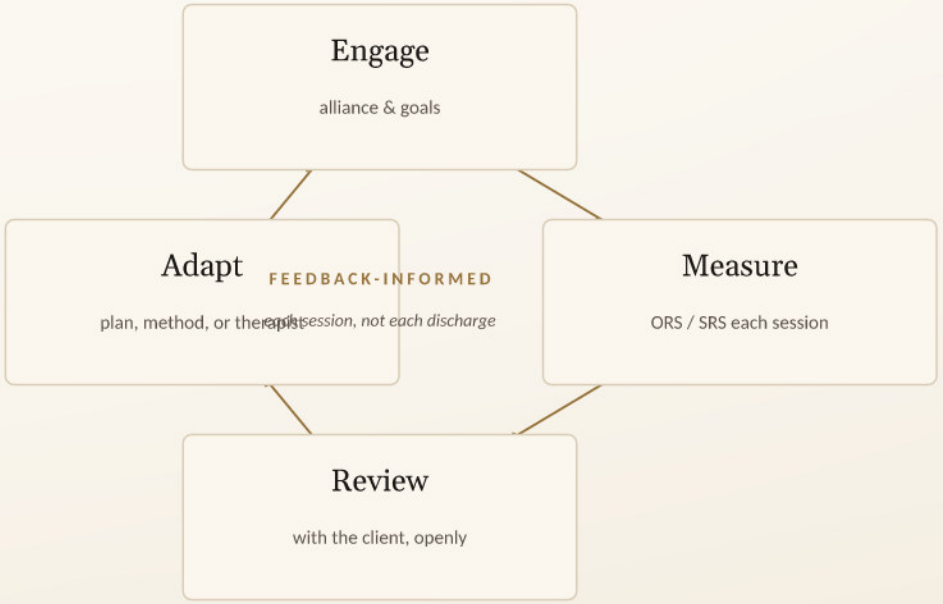
The client's view

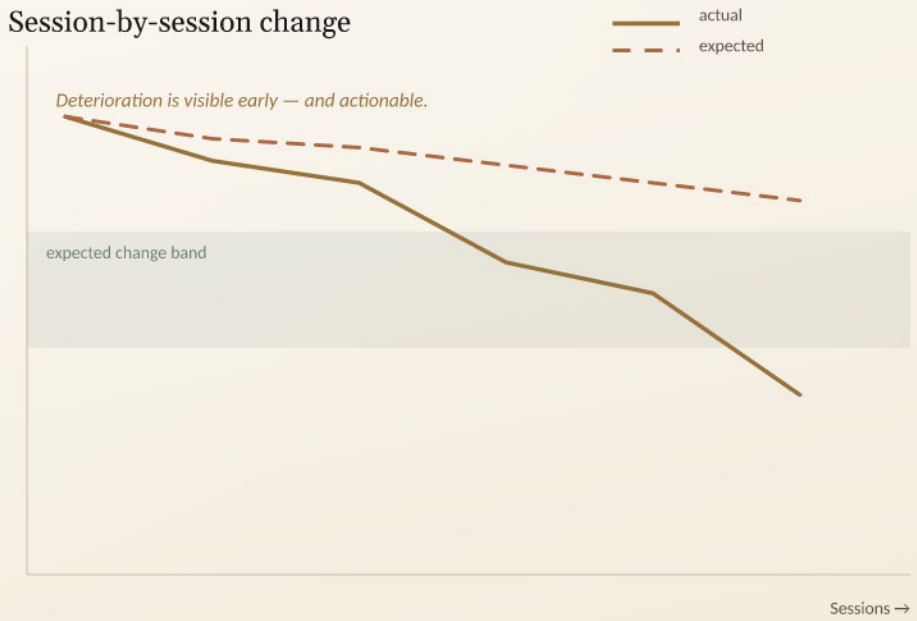
The most predictive feedback is not the therapist's evaluation of the session, but the client's experience of alliance and outcome.

Sprenkle, Davis & Lebow (2009); Wampold & Imel (2015); Duncan et al. (2010).

Feedback as a clinical discipline

Measurement is not surveillance. Done well, it is a structured conversation about whether this work is helping this family.





ORS — Outcome Rating Scale; SRS — Session Rating Scale.

The clinician you are becoming

A closing seminar. Speak from your own experience; listen without advising.

01 Which model feels most like you — and what does that reveal?

02 Where are you most likely to lose your curiosity?

03 What would you want a supervisor to notice about you?

04 What is one commitment you will carry into practicum?

05	SILENT WRITING	10	PAIRS	20 PLENARY	05 CLOSE	MINUTES

A working vocabulary

Terms you should be able to use precisely, in writing and in supervision.

Circular causality

A sequence in which each response becomes the stimulus for the next.

Punctuation

The arbitrary point at which a member says a circular sequence begins.

Homeostasis

The system's tendency to maintain a familiar set point.

Feedback

Information about the effect of an action that modifies subsequent action.

Boundary

The invisible rule regulating closeness and distance between subsystems.

Triangulation

Recruiting a third person to manage tension between two.

Differentiation of self

Staying connected while maintaining one's own thinking.

Enactment

Inviting the family to re-enact a problematic interaction in session.

Genogram

A multigenerational diagram of structure, relationships and patterns.

Joining

The therapist's active accommodation to the family's style and language.

Alliance

The collaborative bond and agreement on goals and tasks.

Externalising

Treating the problem as separate from the person.

Circular questioning

Questions that elicit differences and connections within the system.

Neutrality

A commitment not to align with any one member's account.

Cultural humility

A reflexive, lifelong stance toward one's own cultural location.

Common factors

Change processes shared across therapeutic models.

Reflecting team

A team whose conversation is observed by the family.

Self-of-the-therapist

The clinician's own relational history and reactivity as clinical material.

Core readings

Students are expected to engage the primary texts, not only the summaries in this deck.

01 AAMFT (2015). Code of Ethics. American Association for Marriage and Family Therapy.

02 Bateson, G. (1972). Steps to an Ecology of Mind. Chandler.

03 Becvar, R. J., & Becvar, D. S. (2018). Systems Theory and Family Therapy: A Primer (3rd ed.). Hamilton.

04 Bowen, M. (1978). Family Therapy in Clinical Practice. Jason Aronson.

05 Carr, A. (2019). Family Therapy: Concepts, Process and Practice (3rd ed.). Wiley.

06 de Shazer, S. (1985). Keys to Solution in Brief Therapy. Norton.

07 Duncan, B. L., Miller, S. D., Wampold, B. E., & Hubble, M. A. (Eds.). (2010). The Heart and Soul of Change (2nd ed.). APA.

08 Haley, J. (1976). Problem-Solving Therapy. Jossey-Bass.

09 Johnson, S. M. (2019). Attachment Theory in Practice. Guilford.

10 McGoldrick, M., Gerson, R., & Petry, S. (2008). Genograms: Assessment and Intervention (3rd ed.). Norton.

11 Minuchin, S. (1974). Families and Family Therapy. Harvard University Press.

12 Nichols, M. P., & Davis, S. D. (2020). Family Therapy: Concepts and Methods (12th ed.). Pearson.

13 Satir, V. (1967). Conjoint Family Therapy. Science and Behavior Books.

14 Watzlawick, P., Beavin, J. H., & Jackson, D. D. (1967). Pragmatics of Human Communication. Norton.

15 White, M., & Epston, D. (1990). Narrative Means to Therapeutic Ends. Norton.

16 Wampold, B. E., & Imel, Z. E. (2015). The Great Psychotherapy Debate (2nd ed.). Routledge.

END OF COURSE

The family is not the context of the problem.

It is the context of the solution — and the place where a therapist learns to be useful.

NEXT IN THE PROGRAMME · MFT 702 — THEORIES OF FAMILY THERAPY